

ENGLISH FOR WORK / SEPTEMBER 2026

Dental Assisting English



Teacher's guide

Teach practical workplace communication with specific cases, clear language targets, and a repeatable feedback process.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For Dental assistants and dental support staff.

Fictional English practice, not dental training. Permitted duties, supervision, imaging authorization, and infection-prevention procedures depend on local requirements and qualifications. Refer diagnosis, treatment decisions, informed-consent questions, and aftercare advice to the authorized dental professional.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Your course at a glance

Use the lesson numbers to match this document with the website and the other guides.

01 Clarifying the appointment purpose

Check the appointment mismatch and explain what is confirmed.

02 Room readiness and safe clarification

Name the unresolved check and ask for an authorized pause or alternative.

03 Chairside requests and read-backs

Identify the exact missing word or detail instead of silently guessing.

04 Imaging questions and authorization

Respond to an imaging question by identifying the right decision-maker and preserving the patient's question.

05 Patient comfort, questions, and pauses

Acknowledge the concern and obtain a clinician-led explanation before a decision.

06 Records, identifiers, and factual handoffs

Identify conflicting sources and obtain an authorized record check.

07 Laboratory cases and delivery expectations

Link a delivery risk to a concrete update time and authorized scheduling decision.

08 Aftercare questions and follow-up calls

Identify the unclear instruction and obtain an appropriate authorized response.

Choose your pace

Quick practice: spend 15 minutes reading one conversation and rehearsing one scenario. Full lesson: allow 45-60 minutes for language work, three conversations, two speaking scenarios, and feedback.

Level support

B1 learners can use the frames and a partner. B2 learners can try a response before reading the model. C1 learners can add the harder follow-up and reduce preparation time. These are teaching suggestions, not certified CEFR level ratings.

After practicing: open the AI prompt workshop to choose a vocabulary, grammar, role-play, or new-dialogue activity.

A 60-minute lesson that works

Prepare the case and the learner pages. Keep the model response hidden until learners have attempted their own. Choose two field terms that are important to understanding the case.

0-5 minutes | Activate experience

Ask learners to name a comparable situation without sharing confidential details. Introduce the communication goal.

5-12 minutes | Read and sort facts

Read the case. Learners identify confirmed facts, unknowns, the audience, and the immediate communication need.

12-22 minutes | Notice and practice language

Explain the workshop pattern. Work through the vocabulary and editing example, then discuss both language checks and the reasons behind the choices.

22-35 minutes | 05 • Conversations

Read one ten-turn conversation aloud, notice a useful expression, then compare the other two. Identify each speaker's purpose and how the exchange ends.

35-47 minutes | 06 • Say it

Rehearse both speaking scenarios from the workbook. Give two minutes of preparation, switch roles, and use the success checks. Add the complication only after a successful first round.

47-55 minutes | Compare and revise

Reveal the model. Discuss alternative acceptable wording. Give one meaning correction and one language correction, then repeat.

55-60 minutes | Retrieve and transfer

Close the materials. Learners recall two expressions and identify one communication habit to use in another situation.

Adapt the session

For 45 minutes, study one complete conversation and rehearse one scenario; assign the others for oral follow-up. For 90 minutes, rehearse all three conversations and give both scenarios a second round. More time should produce more meaningful practice, not longer explanations.

LESSON 01 / FACILITATOR NOTES

Clarifying the appointment purpose

Check the appointment mismatch and explain what is confirmed.

CASE

A fictional patient arrives expecting a filling today. The schedule shows a thirty-minute consultation, and no procedure is confirmed. The dentist can clarify the purpose before the patient decides whether to continue.

Teach this move

'Consultation' and 'procedure' are not interchangeable. Describe the booking you can see, acknowledge the patient's understanding, and ask the authorized person to clarify without blaming the patient or promising treatment.

Vocabulary to check: consultation, procedure, appointment slot, treatment plan.

Model response

I see a thirty-minute consultation on today's schedule, but you were expecting a filling. I have not confirmed a procedure for this visit. Let me check with the dentist and reception before we describe what will happen today. Please tell us what you understood from the earlier booking conversation.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revision names the mismatch without dismissing the patient's expectation.

Second-round challenge

Reception finds a note saying 'discuss filling options.' Explain why that note does not by itself confirm that a filling will be performed.

Language-check answers

1. D - consultation Correct. The schedule confirms the consultation only.
2. A - check that Correct. An open check resolves the mismatch without promising treatment.

05 • Conversations

1. I thought the treatment was today - At check-in, a patient and assistant discover different expectations.

2. Adding a concern to an existing slot - An assistant and receptionist coordinate a new question during a scheduled visit.

3. Which visit does the reminder refer to? - An assistant helps a patient distinguish two upcoming appointments.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | A transport booking depends on the finish time

The patient needs to book a ride home. The schedule reserves thirty minutes for a consultation, but the dentist has not confirmed the likely finish time. Reception can give updates after clarification.

Role A: Assistant: distinguish the reserved slot from a guaranteed departure time.

Role B: Patient: you must tell the transport company when to collect you and can request a flexible pickup.

Possible opening: Can I promise the driver I'll be outside exactly thirty minutes after the appointment starts?

Success checks

- Explain what the appointment slot does and does not establish.
- Avoid guaranteeing a finish time outside your authority.
- Identify who can clarify timing and how the patient will receive an update.

Add a complication: The patient asks you to guarantee that no additional discussion will be needed.

LESSON 02 / FACILITATOR NOTES

Room readiness and safe clarification

Name the unresolved check and ask for an authorized pause or alternative.

CASE

A fictional instrument pack is present, but its required readiness documentation cannot be confirmed. The dentist is about to start. You are not authorized to waive the practice's checks. A verified replacement may be available through the lead assistant.

Teach this move

A room can look tidy without every required readiness check being complete. This lesson practices escalation language, not sterilization methods. Describe the missing confirmation accurately; do not label equipment contaminated without evidence or bypass a required check.

Vocabulary to check: operatory, instrument tray, sterilization record, turnover.

Model response

Before we start, I need to flag a readiness discrepancy. I cannot confirm the required record for this pack, so I am not treating it as ready for use. Could we hold this setup while the lead assistant follows the approved verification process or provides a verified replacement? I will report the status as soon as it is confirmed.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. Appearance does not replace the required confirmation, and the revised request identifies the action needed.

Second-round challenge

A colleague says the pack looks identical to a verified one. Explain why visual similarity does not answer the missing-record question.

Language-check answers

1. B - unverified Correct. This accurately states the missing confirmation.
2. B - hold Correct. 'Hold' asks for a pause pending confirmation.

05 • Conversations

1. The record is not confirmed - An assistant raises a readiness concern before the fictional appointment begins.

2. Ready for which appointment? - Two assistants clarify a room-status message before the next patient is brought in.

3. A name on the tray list - An assistant clarifies a tray request rather than guessing between similar labels.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | A rushed room-status message

Reception asks whether room three is ready. Turnover is complete, but the appointment-specific setup check is still pending. The lead assistant expects an update in five minutes but has not approved room use.

Role A: Assistant: distinguish completed turnover from pending setup and request that reception wait for confirmation.

Role B: Receptionist: you want to reduce the patient's wait and thought 'cleaned' meant fully ready.

Possible opening: The room has been cleaned, so can I bring the patient in now?

Success checks

- Name the completed stage and the pending stage.
- Request a specific pause without claiming contamination or danger not established by the facts.
- Confirm who will communicate the final readiness status.

Add a complication: The next patient has arrived early. Their arrival does not change the required readiness process.

LESSON 03 / FACILITATOR NOTES

Chairside requests and read-backs

Identify the exact missing word or detail instead of silently guessing.

CASE

During a fictional setup briefing, you hear 'tray' but miss the rest because another conversation interrupts. The list includes both an instrument tray and an impression tray. You have not selected either. Ask which tray is intended and read back the request before continuing under the practice's procedures.

Teach this move

A concise read-back is useful when names, numbers, or locations could be confused. This is communication practice, not instruction in using dental instruments. Ask about the uncertain part, repeat the answer, and let the clinician correct it.

Vocabulary to check: chairside, suction, retraction, read-back.

Model response

I heard 'tray,' but I missed which kind because of the other conversation. The list includes both an instrument tray and an impression tray. Which one did you mean? I'll repeat the full name to check that I have understood before continuing with the setup.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The listener can repair a specific gap more easily than an unspecified uncertainty.

Second-round challenge

The dentist uses 'that one' while looking away. Ask for an explicit description rather than relying on a direction you cannot see.

Language-check answers

1. C - obscured Correct. 'Obscured' means it was made unclear or difficult to hear.
2. D - read that back Correct. Repeating the detail checks shared understanding.

05 • Conversations

1. The last word was lost - A noisy setup discussion makes an item name unclear.

2. The fault belongs to operatory two - An assistant and lead assistant clarify a suction-equipment report before a room is described as ready.

3. A request made too quickly - A trainee assistant requests a clearer pace during a room briefing.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | An unfamiliar room abbreviation

At this fictional practice, a message says 'check OP4.' You do not know the local abbreviation. The lead assistant means checking the setup list for operatory four, not declaring the room ready. No readiness check has been completed.

Role A: Assistant: ask what OP4 means and clarify which check is requested.

Role B: Lead assistant: in this practice OP means operatory. You need the setup list checked for operatory four; report any missing information to you.

Possible opening: Could you check OP4 before our room briefing?

Success checks

- Ask for the abbreviation to be expanded without guessing.
- Read back the room number and the requested check.
- Distinguish checking a list from confirming overall room readiness.

Add a complication: A second message mentions OP2. It concerns a separate appointment; clarify which room belongs to the current request.

LESSON 04 / FACILITATOR NOTES

Imaging questions and authorization

Respond to an imaging question by identifying the right decision-maker and preserving the patient's question.

CASE

A fictional patient asks whether an image proves that a tooth needs treatment. You are assisting with the visit and are not authorized to interpret the image. The dentist has not yet discussed it with the patient.

Teach this move

Recognizing the word 'radiograph' does not qualify someone to interpret an image. Nor does being asked for a retake establish authority to perform it. This lesson covers communication only; follow local requirements and the current authorized order.

Vocabulary to check: radiograph, imaging order, retake, authorization.

Model response

The dentist needs to interpret the image and explain what it means for your care. I am not authorized to tell you whether it proves treatment is needed. I can let the dentist know you want that question answered before any decision is made, and help make sure you have an opportunity to ask it.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revised wording avoids an unauthorized diagnosis or treatment conclusion.

Second-round challenge

The patient says another assistant once explained a similar image. Keep the boundary about your own role without criticizing that person.

Language-check answers

1. B - interpret Correct. Interpreting the image belongs with the qualified clinician.
2. C - order Correct. The authorized order identifies the requested imaging.

05 • Conversations

1. Does the image prove it? - A patient asks an assistant for a clinical interpretation.

2. A possible retake is not an order - An assistant clarifies a passing comment about imaging.
3. Explaining a word, not a finding - A patient asks about terminology on an appointment note.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | A copied order description

A colleague suggests copying an imaging description from a previous visit into today's setup message. Today's authorized order has not yet been verified. You do not know whether the earlier description still applies.

Role A: Assistant: explain why a previous entry cannot substitute for a current authorized request.

Role B: Colleague: you want to save time, but can ask the dentist to confirm today's order.

Possible opening: Can we use the same imaging description as last time to get the room moving?

Success checks

- Distinguish historical information from a current order.
- Ask the authorized person for the current request.
- Avoid advising on whether imaging is clinically necessary.

Add a complication: The patient remembers the earlier image too, but that memory does not establish today's request.

LESSON 05 / FACILITATOR NOTES

Patient comfort, questions, and pauses

Acknowledge the concern and obtain a clinician-led explanation before a decision.

CASE

A fictional patient is anxious and asks you to promise a procedure will not hurt. The dentist has not yet discussed comfort measures or the patient's concerns. You can help communicate the concern and ask the clinician to explain the plan and an appropriate pause signal.

Teach this move

'Don't worry' can sound dismissive when a patient has a specific concern. Name the concern, be honest about what you cannot promise, and support the patient's opportunity to ask the authorized professional. This exercise does not teach consent law or pain management.

Vocabulary to check: patient concern, pause signal, informed consent, reassurance.

Model response

I understand you are worried about discomfort. I cannot promise exactly how a procedure will feel for you. Before anything begins, let's ask the dentist to explain the comfort plan and how you can signal if you need attention or a pause. I will make sure your concern is passed on clearly.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revised reassurance offers a practical next step without predicting the patient's experience.

Second-round challenge

The patient asks whether asking for a pause will annoy the team. Support the question and ask the clinician to establish the communication plan.

Language-check answers

1. C - guarantee Correct. The assistant cannot guarantee an individual experience.
2. D - explanation Correct. The patient is asking to understand more before deciding.

05 · Conversations

1. A promise the assistant cannot make - An anxious patient asks about comfort before the clinician explains the proposed care.
2. A question behind the silence - An assistant checks whether a quiet patient has an unresolved concern.
3. The companion answers for the patient - An assistant protects space for the patient's own question.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | A question after the explanation

The dentist has explained a proposed procedure, but the patient tells you they still do not understand one term. No decision is established by this scenario. You can ask the dentist to revisit the explanation.

Role A: Assistant: acknowledge the unresolved term and help the patient obtain clarification.

Role B: Patient: you nodded because you felt embarrassed, not because you understood.

Possible opening: I nodded earlier, but I don't actually know what that term means for me.

Success checks

- Distinguish hearing an explanation from understanding it.
- Avoid labeling the patient's earlier nod as a decision.
- Bring the specific question back to the authorized clinician.

Add a complication: A colleague says the explanation has already been given. You still need to communicate that understanding remains unresolved.

LESSON 06 / FACILITATOR NOTES

Records, identifiers, and factual handoffs

Identify conflicting sources and obtain an authorized record check.

CASE

Two fictional patients share a surname. The appointment list and a folder label contain a mismatch in another identifier. You have not used the folder. The lead assistant can check the records through the approved process.

Teach this move

A familiar surname does not resolve a mismatch. Keep the versions distinct, protect private details, and use the approved correction process. Practice with fictional identifiers only.

Vocabulary to check: patient identifier, clinical chart, discrepancy, amendment.

Model response

Before this folder is used, I need to flag an identifier mismatch between the appointment list and its label. Two patients share the surname, so that name alone does not resolve it. I have held the folder and need the approved identity check. Please confirm the correct record before the team proceeds.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. One matching detail does not resolve conflicting information.

Second-round challenge

A colleague recognizes the patient from a previous visit. Explain why recognition does not remove the record mismatch.

Language-check answers

1. D - discrepancy Correct. A discrepancy is a mismatch needing a check.
2. A - amendment Correct. An amendment is an authorized record correction or addition.

05 · Conversations

1. The surname matches, but the record does not - An assistant reports a discrepancy before a folder is used.
2. Whose words are in the note? - A chart phrase may be an interpretation rather than a quotation.
3. A private record in a public space - An assistant redirects a records discussion away from a reception queue.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 • Say it: second scenario | A copied note with the wrong date

A draft handoff includes a previous visit's date. Today's date is verified, but you cannot silently overwrite finalized entries. The records lead handles corrections.

Role A: Assistant: flag the discrepancy and ask whether the entry is finalized.

Role B: Records lead: determine the entry's status and direct the approved correction process.

Possible opening: This handoff appears to include the previous visit's date; can we check its status before use?

Success checks

- Name the mismatch and its source.
- Do not hide or silently overwrite a finalized record.
- Confirm the correction route and notify the appropriate recipient.

Add a complication: A colleague asks you to erase the old entry so nobody sees the mistake.

LESSON 07 / FACILITATOR NOTES

Laboratory cases and delivery expectations

Link a delivery risk to a concrete update time and authorized scheduling decision.

CASE

A fictional laboratory estimates a case will arrive Tuesday by noon; the patient's appointment is at 14:00. Delivery is not guaranteed, and the dentist must confirm readiness before the appointment proceeds. The lab can provide a status update Monday at 16:00.

Teach this move

Laboratory dispatch, delivery, and clinical readiness are different stages. An assistant can coordinate information without changing the prescription, choosing a shade, or promising that a restoration will be suitable before the clinician checks it.

Vocabulary to check: laboratory prescription, impression, shade, turnaround time.

Model response

The appointment is Tuesday at two, and your current delivery estimate is noon that day. That leaves little time for the dentist's readiness check, so I cannot treat the appointment as secured by this estimate alone. Could you update us Monday at four? We will then ask the authorized team to confirm the scheduling decision.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revision separates logistics from an authorized clinical readiness decision.

Second-round challenge

Reception asks whether dispatch means the case is ready for the patient. Explain the remaining delivery and clinical checks.

Language-check answers

1. C - estimate Correct. An estimate preserves the laboratory's stated uncertainty.
2. A - authorized clinician Correct. The assistant should refer the specification to the authorized clinician.

05 • Conversations

1. Noon is an estimate - An assistant and laboratory coordinator discuss a tight delivery window.

2. An unclear shade instruction - A laboratory query requires the clinician's clarification rather than an assistant's guess.

3. Delivered is not clinically approved - An assistant explains the status of a returned laboratory case to reception.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | A second parcel with a similar reference

Two lab parcels arrive with similar case references. One patient is due at 14:00. The identifiers on one parcel need clarification, and the authorized team has not approved either case for use.

Role A: Assistant: ask the lab to verify the reference through the approved private channel.

Role B: Lab coordinator: you can check dispatch records but cannot make the dentist's clinical readiness decision.

Possible opening: Before we associate this parcel with today's appointment, can you confirm the full case reference?

Success checks

- Verify the specific case instead of assuming from timing.
- Protect private identifiers during the query.
- Keep receipt, identity verification, and clinical approval distinct.

Add a complication: Reception wants to rely on the expected arrival date because the patient is on the way.

LESSON 08 / FACILITATOR NOTES

Aftercare questions and follow-up calls

Identify the unclear instruction and obtain an appropriate authorized response.

CASE

A fictional patient calls with an unclear phrase in their written postoperative instructions. You are not authorized to reinterpret or change those instructions. The clinician can answer through the practice's callback process. If the caller reports an urgent or emergency concern, use the designated route rather than an ordinary queue.

Teach this move

This is language practice, not symptom assessment or treatment guidance. Do not recommend doses, label symptoms normal, or promise a response time that has not been agreed. Use the practice's screening and escalation procedures; do not substitute this lesson for them.

Vocabulary to check: postoperative instructions, review appointment, callback, urgent concern.

Model response

Please tell me which phrase in the written instructions you want clarified. I can pass your question to the authorized clinician, but I should not reinterpret or change the guidance myself. We will use the appropriate contact process for your concern, and any urgent issue must follow the practice's designated urgent or emergency route.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revision avoids unsupported reassurance and guessing at clinical guidance.

Second-round challenge

The patient asks whether a missed callback means the question is unimportant. Explain that a communication delay does not establish clinical importance and use the appropriate follow-up route.

Language-check answers

1. A - authorized Correct. The appropriate clinician must clarify or change the guidance.
2. C - callback Correct. A callback is a return contact, not automatically an appointment.

05 • Conversations

1. The written phrase is unclear - A fictional aftercare sheet says 'resume your usual routine.' No interpretation is supplied. The patient has a wording question, and no urgent concern has been reported.
2. An overdue callback - An assistant follows up on an unanswered patient question.
3. Review visit or phone contact? - A patient checks two different follow-up arrangements.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 • Say it: second scenario | Advice remembered from a previous procedure

The patient asks whether instructions from an earlier procedure can replace the current written guidance. You have not verified any equivalence and are not authorized to change instructions. The appropriate clinician can clarify through the designated process.

Role A: Assistant: explain why prior guidance cannot simply be assumed to apply.

Role B: Patient: you remember the earlier wording more clearly and want to know which guidance is current.

Possible opening: Last time the instructions were different. Should I just follow the old ones again?

Success checks

- Distinguish previous instructions from current guidance.
- Do not choose, reinterpret, or invent clinical advice.
- Route the question using the appropriate regular or urgent process.

Add a complication: The patient asks you to choose the most convenient version before the clinician responds.

Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and switches roles and repeats the exchange. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

Use the language responsibly

Fictional English practice, not dental training. Permitted duties, supervision, imaging authorization, and infection-prevention procedures depend on local requirements and qualifications. Refer diagnosis, treatment decisions, informed-consent questions, and aftercare advice to the authorized dental professional.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

BLS: Dental Assistants

<https://www.bls.gov/ooh/healthcare/dental-assistants.htm>

CDC: Plain Language

<https://www.cdc.gov/health-literacy/php/develop-materials/plain-language.html>

NIDCR: Dental Materials

<https://www.nidcr.nih.gov/grants-funding/funded-research/research-investments-advances/dental-materials>

National Dental Practice-Based Research Network: Impression Techniques for Single-Unit Crowns

<https://pmc.ncbi.nlm.nih.gov/articles/PMC5794617/>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.